

ITCH WITH NO RASH

Check if the patient needs urgent attention → 67.

- Confirm there is no rash, especially scabies, lice or insect bites.
 - If generalised itchy rash →69.
 - If localised itchy rash →70.
- If itch around anus only →48.

Is the skin very dry?

Yes

Dry skin (xeroderma /ichthyosis) likely

No

Did the patient start any new medications in the weeks before the itch started?

Yes

Medication side-effect likely

- Continue the medication only if still necessary.
- Advise to return if rash develops or itch persists.

No

- If yellow skin/eyes, **jaundice** likely →79.
- If itch persists > 2 weeks:
 - Test for anaemia → 27, HIV → 110 and diabetes → 17.
 - Check CRP, creatinine (eGFR), ALT and TSH.
 - Refer to doctor.

- Advise to:
 - Avoid hot baths, wool/itchy fabrics and scratching as these may worsen itch.
 - Wash with **aqueous cream (UEA)** instead of soap. Avoid using aqueous cream as moisturiser (emollient).
 - Moisturise skin with **emulsifying ointment (UE)** twice a day.
 - Avoid scrubbing the skin and washing more than once a day. Gently pat skin dry.
 - Keep nails short.
- If severe itch, give **chlorphenamine** 4mg at night, or up to 6-8 hourly for up to 5 days.
- If known with a life-limiting illness, consider giving palliative care → 170.
- If no better, discuss/refer.

If diagnosis uncertain, discuss/refer.